

REFERRAL COVER SHEET

Chesapeake Regional Medical Associates
4801 Kenilworth Ave, Suite 12, Riverdale, MD 20737
Tel: (301) 864-2200 Fax: (301) 864-2219

TO:

Johns Hopkins Hospital – Neurology Dept (Outpatient)
Attn: Scheduling / Neurology Referrals
Fax: (410) 955-2987

FROM:

Dr. Anita Patel, MD (NPI: 1932847561)
Internal Medicine / Primary Care

DATE TRANSMITTED: 11/14/2024 **TIME:** 09:22

PATIENT:

Gerald T. Hoffmann DOB: 03/17/1956 MRN: CRMA-00447821

REASON FOR REFERRAL:

Episodic neurological symptoms – ? etiology
Pt with recurrent episodes of R arm weakness, some H/A component
Rule out migraine variant vs other. Neurology f/u requested.
Please evaluate and advise.

URGENCY: ☐ Emergent ☒ Urgent ☐ Routine

INSURANCE:

Aetna PPO – Member ID: W44901872 Group: 87441
Secondary: Medicare Part B HIC#: 2KH-GH9-WW04A

DOCUMENTS ENCLOSED:

☒ Office notes (x4)	☒ Labs
☒ Imaging reports	☒ Med list
☒ Discharge summary	☐ Prior auth

*** NOTE: Some documents may be duplicated due to EHR export error – apologies ***
Please call our office if any pages are missing or illegible.

Anita Patel, MD
Referring Provider Signature

Date: 11/14/2024

PATIENT DEMOGRAPHICS / REGISTRATION

Chesapeake Regional Medical Associates – Administrative Record

Last Name	Hoffmann
First Name	Gerald
Middle Initial	T.
Date of Birth	03/17/1956 (Age: 68)
Sex	Male
SSN (last 4)	****-**-9341
MRN (CRMA)	CRMA-00447821

Address	7714 Millstream Court, Bowie, MD 20715
Phone (Home)	(301) 805-4412
Phone (Cell)	(301) 219-8863
Email	ghoffmann56@comcast.net
Emergency Contact	Diane Hoffmann (wife) – (301) 219-0044

INSURANCE:

Primary	Aetna PPO
Member ID	W44901872
Group	87441
Secondary	Medicare Part B
HIC #	2KH-GH9-WW04A
Effective Date	01/01/2021
PCP on file	Dr. Anita Patel, MD

ALLERGIES (per registration):

Penicillin – rash
Sulfa drugs – unknown reaction
NKDA otherwise per pt report (note: allergy list differs from pharmacy – see med rec)

PREFERRED PHARMACY:

CVS #1042 – 4900 Annapolis Rd, Bladensburg MD 20710
Ph: (301) 864-0011

*** Registration last updated: 09/05/2024 by front desk staff (L.Wren) ***
*** Insurance eligibility NOT verified at time of this fax ***

PROGRESS NOTE — OFFICE VISIT

Chesapeake Regional Medical Associates

Provider: Anita Patel, MD NPI: 1932847561

Date of Service: 11/12/2024 Visit Type: Established Patient — Acute

Signed: 11/13/2024 08:44

PATIENT: Hoffmann, Gerald T. DOB: 03/17/1956 MRN: CRMA-00447821

REASON FOR VISIT:

Pt presents c/o episode of right arm weakness that occurred approx 3 days ago (11/09/2024). He describes waking up and noticing his right arm felt "heavy" and "not working right" — had difficulty lifting coffee cup, buttoning shirt. No pain.

Lasted "maybe 20-25 minutes" then fully resolved. Pt initially attributed to sleeping

on arm wrong. Wife encouraged him to come in.

HPI:

68 yo M with hx of HTN, HLD, T2DM (diet controlled), former smoker (quit 2011), and intermittent "sinus headaches" per prior records. Reports 1-2 prior episodes over past 8 months of similar transient R arm/hand weakness, each lasting <30 min,

fully resolving. One of these was assoc with mild HA — described as pressure, right-sided, w/o nausea. Pt seen in urgent care (see enclosed UC note) for one episode, was told possibly "tension headache." Never formally worked up.

Denies: leg weakness, facial droop, visual changes (though reports "eyes felt funny" x1 — pt vague, unable to elaborate), dysphagia, dysarthria, vertigo, loss of consciousness, chest pain, palpitations.

Family history notable for father w/ stroke at age 72. Mother: DM, HTN.

Brother: MI at 58.

MEDICATIONS: (per pharmacy reconciliation — see attached list)

Lisinopril 10mg QD, Atorvastatin 40mg QHS, Metformin 500mg BID (on hold per pt — stopped taking "months ago"), Aspirin 81mg QD (pt reports taking "sometimes"), Amlodipine 5mg QD (added 2023), Omeprazole 20mg QD, Loratadine 10mg QD PRN, Vitamin D3 2000IU QD

PHYSICAL EXAM:

Vitals: BP 158/94 (L arm), HR 82 reg, RR 16, SpO2 97% RA, Temp 98.3F, Wt 214 lbs

Gen: Alert, oriented, appears stated age, no acute distress

HEENT: NC/AT, MMM, no JVD

CV: RRR, no m/r/g

Pulm: CTA bilaterally

Abd: Soft, NT/ND

Neuro (LIMITED EXAM):

- MS: Alert, conversant, speech fluent
- CN: Pupils equal reactive, EOMI (not formally tested today), face symmetric
- Motor: 5/5 bilateral upper and lower extremities TODAY — no deficits at time of exam (symptoms fully resolved)
- Sensation: Grossly intact
- Coordination: not formally tested
- Reflexes: not formally tested

ASSESSMENT:

1. Episodic right arm weakness — etiology unclear; differential includes: migraine w/ motor aura (hemiplegic migraine variant?), focal seizure,

demyelinating process, or vascular etiology given risk factor burden.

Note: no prior neuro w/u.

2. Hypertension — poorly controlled (BP today elevated, see above)
3. HLD — on statin
4. T2DM — patient non-adherent with metformin
5. Hx sinus headache / possible migraine — not formally diagnosed

PLAN:

- Neurology referral placed (Hopkins preferred per pt) — see referral form
- Ordered: BMP, HbA1c, fasting lipid panel, CBC — results attached
- EKG ordered today — results pending at time of this note
- Encouraged strict BP medication compliance; counseled on diet
- Advised pt to return or go to ED if symptoms recur
- Will hold on further w/u pending neurology input

Electronically signed: A. Patel MD 11/13/2024 08:44

[AUTO-GENERATED NOTE — EXPORTED FROM MEDEXTEND EHR v4.2.1]

URGENT CARE VISIT NOTE — PRIOR ENCOUNTER

FastCare Urgent Care — Bowie, MD
8220 Race Track Rd, Bowie MD 20715 Ph: (301) 464-7100
Provider: James R. Okafor, MD
Date: 08/22/2024 Time: 2:14 PM
Chart printed 11/12/2024 for release per pt request

PT: Hoffmann, Gerald **DOB:** 3/17/56 **Chart#:** FC-209941

CC: "My right hand went numb and my arm felt weak this morning"

HPI:

68M presenting with episode of right upper extremity paresthesia and weakness onset approx 0730, spontaneously resolved by 0800. Pt reports awakening and noticing right hand felt "asleep" and arm heavy; states he couldn't close his hand properly. No preceding trauma or unusual sleeping position noted by wife. Associated mild bifrontal headache. No nausea, no visual symptoms, no vertigo. No speech difficulty. No similar episodes in remote past per pt (NOTE: pt's wife interjected that this had happened once before "a few months ago" and resolved quickly — pt minimized this).

PMHx: HTN, HLD, prediabetes vs T2DM (pt unclear), former smoker
Meds: Lisinopril, atorvastatin, "a water pill" (unable to confirm — not on med list), aspirin occasionally
Allergies: PCN

EXAM:

BP 162/96 (R), HR 78, RR 14, SpO2 98%, T 98.1
GEN: Well appearing, conversational, NAD
NEURO: Grossly intact at time of exam — no focal deficits. Speech clear.
Face symmetric. Upper extremity strength 5/5 bilaterally.
(Symptoms had resolved approx 4 hours prior to presentation)

EKG: (performed in office)

NSR, rate 80, normal axis, no ST changes, no LVH pattern, no afib noted

IMPRESSION:

1. Episodic right arm numbness/weakness, resolved — possibly tension-type headache with associated autonomic symptoms vs. peripheral nerve compression (thoracic outlet? ulnar?). Less likely central etiology given age of resolution and headache assoc.
2. Elevated BP — likely chronic, patient counseled

PLAN:

- Advised patient to follow up with PCP urgently for further evaluation
- Ibuprofen 400mg for headache (given at UC — pt tolerated well)
- RETURN PRECAUTIONS given: facial droop, speech difficulty, inability to walk, persistent weakness — go to ED immediately
- No imaging obtained at this visit

** NOTE: Pt expressed reluctance to seek further care — "I feel fine now" **

Signed: J.R. Okafor MD 08/22/2024 14:58
[Document faxed from FastCare — image quality may be degraded]

CARDIOLOGY CONSULTATION NOTE — PRIOR ENCOUNTER

Chesapeake Heart Group, LLC

Provider: Dr. Sameer Gupta, MD, FACC

Date of Service: 04/11/2024 Signed: 04/12/2024

Reason for consult: HTN mgmt / risk stratification

Pt: Hoffmann, Gerald T. DOB: 03/17/1956

HISTORY OF PRESENT ILLNESS:

Mr. Hoffmann is a 67 yo M (now 68) referred by Dr. Patel for evaluation of suboptimally controlled hypertension and cardiovascular risk assessment. Patient has documented hx of HTN x approx 10 years, currently on lisinopril 10mg and amlodipine 5mg. Reports compliance "most of the time." Home BPs logged range 140s-160s systolic per patient report. Denies chest pain, dyspnea on exertion, palpitations, syncope, orthopnea, edema.

Also endorses occasional right-sided headaches, which he attributes to sinuses. Does not describe visual prodrome or neurological symptoms (pt and wife both deny at this visit — note this differs from UC note from 08/2024). No prior cardiac events.

PMHx: HTN, HLD (LDL 118 on statin — see labs 02/2024), T2DM (A1c 7.1 as of 02/2024 — per PCP records), former smoker (30 pack-yr hx, quit 2011)

FHx: Father — CVA at 72. Brother — MI at 58. Mother — DM.

MEDICATIONS (per consult intake — may differ from current list):

Lisinopril 10mg QD, Amlodipine 5mg QD, Atorvastatin 40mg QHS,
Aspirin 81mg QD, Metformin 500mg BID, Omeprazole 20mg QD
Vitamin D 2000IU QD, Loratadine 10mg PRN
(pt also mentioned taking fish oil — not on Rx list)

EXAM:

BP 154/90 (L) / 152/88 (R) — bilateral arm difference < 5mmHg
HR 76, regular. RR 14. SpO2 98%.
Wt: 217 lbs. BMI 29.4.
CV: RRR, S1S2 normal, no murmurs. No S3/S4.
Pulm: CTAB.
Ext: No edema. Pulses 2+ bilaterally.

STUDIES REVIEWED:

Echo (02/2024): EF 58%, mild LVH, no wall motion abnormalities, no valvular pathology of significance. LA mildly enlarged (4.2cm).
Lipid panel (02/2024): TC 198, LDL 118, HDL 42, TG 190
EKG (today): NSR, rate 74. LVH criteria met by voltage. No ischemic changes.
No prior Holter on file.

IMPRESSION / RECOMMENDATIONS:

1. HTN — suboptimally controlled on current regimen. Increase lisinopril to 20mg QD. Consider adding HCTZ 12.5mg if targets not met at follow up.
2. LVH on echo — likely hypertensive. Optimize BP control.
3. CV risk — intermediate to high risk given DM, HTN, HLD, smoking hx, family history. Recommend continue statin, aspirin. Discussed lifestyle mod.
4. LA enlargement — monitor. Low clinical suspicion for afib at this time but would consider ambulatory monitoring if clinical change.
5. Headaches — not assessed at this visit; deferred to PCP.

Follow up in 3 months. Lipids, BMP at next visit.

Patient educated on DASH diet and sodium restriction.

Sameer Gupta, MD, FACC Signed: 04/12/2024 10:17
Chesapeake Heart Group — (301) 805-7700
[Copy forwarded to Dr. Patel — original in CRMA chart]

ANNUAL WELLNESS VISIT — PRIOR ENCOUNTER

Chesapeake Regional Medical Associates

Provider: Anita Patel, MD

Date of Service: 02/06/2024 Signed: 02/08/2024

[Exported from MedExtend EHR v4.2.1 — full chart export 11/12/2024]

Pt: Hoffmann, Gerald T. DOB: 3/17/56 MRN: CRMA-00447821

***** COPY FORWARD — ANNUAL VISIT NOTE *****

[Note: The following history section was carried forward from the 2023 annual visit note and may not fully reflect interval changes — please verify]

ANNUAL HEALTH MAINTENANCE:

Colonoscopy: 2019 — 1 polyp removed, benign; repeat in 5yr (due 2024 — pt declined this year, to reschedule)

PSA: 1.8 (2023) — within normal limits; recheck ordered

Influenza vaccine: administered 10/2023

Pneumococcal (PPSV23): given 2022

Shingrix x2: completed 2022

PROBLEM LIST (active as of 02/2024):

- Essential HTN
- Hyperlipidemia
- Type 2 DM — diet controlled (A1c 7.1) [NOTE: per 04/2024 cardiology note, pt was also on Metformin — discrepancy noted, unclear when started]
- Seasonal allergic rhinitis
- Chronic low back pain — stable, not on narcotics
- GERD — on omeprazole
- Former tobacco use (quit 2011 — 30 pk/yr hx)
- Obesity (BMI 29.4 — borderline)
- Anxiety NOS — patient declines therapy or pharmacotherapy at this time
- Hx: R knee arthroscopy 2009 (meniscal tear)
- Hx: cholecystectomy 2014 (laparoscopic)

ROS: (Selected — full ROS in chart)

Positive: occasional right-sided headaches ("sinus pressure"), low back pain, mild fatigue

Negative: chest pain, SOB, palpitations, visual changes, weakness, numbness, speech changes, bowel/bladder dysfunction

[Note: pt denied neurological symptoms at THIS visit — contrast with UC note 08/2024 re: arm weakness episode wife reports predated this visit]

VITALS:

BP 148/88 (L), HR 74, RR 14, SpO2 99%, T 97.9F, Wt 219 lbs

EXAM:

GEN: WDNW male, NAD

HEENT: Oropharynx clear, TMs clear b/l, sinuses non-tender

CV: RRR, no m/r/g

PULM: CTA

Abd: Soft, well-healed scar RUQ, NT, no organomegaly

MSK: Mild lumbar paraspinal tenderness

Neuro: Grossly intact — cranial nerves II-XII intact, strength 5/5 all extremities,
sensation intact, gait normal

LABS (02/06/2024 — fasting):

HbA1c: 7.1 Glucose: 128 (fasting)

BMP: Na 139, K 4.2, Cl 102, CO2 25, BUN 18, Cr 1.0, eGFR >60
CBC: WBC 7.2, Hgb 14.8, Plt 224 - unremarkable
Lipids: TC 198, LDL 118, HDL 42, TG 190
TSH: 2.3 (normal)
PSA: 1.8

ASSESSMENT / PLAN:

1. HTN - continue current regimen, goal <130/80; cardiology referral placed
2. HLD - LDL above goal, continue atorvastatin 40mg
3. DM - A1c 7.1, discussed need for medication compliance
4. Headaches - likely sinusogenic; loratadine, nasal rinse recommended
[NB: no formal migraine workup; pt did not report neurological features]
5. GERD - continue omeprazole
6. Anxiety - watchful waiting, patient preference
7. Preventive health - as noted above

A. Patel MD - Signed 02/08/2024 07:52
[END COPY FORWARDED NOTE]

LABORATORY & IMAGING RESULTS

Chesapeake Regional Medical Associates / LabCorp Reference

Patient: Hoffmann, Gerald T. DOB: 3/17/56 MRN: CRMA-00447821

LABS — Drawn 11/12/2024 (per Dr. Patel order):

TEST	RESULT	REF RANGE	FLAG	UNIT
Sodium	138	136-145		mEq/L
Potassium	4.1	3.5-5.0		mEq/L
Chloride	101	98-107		mEq/L
CO2	24	22-29		mEq/L
BUN	21	7-25		mg/dL
Creatinine	1.1	0.7-1.2		mg/dL
eGFR	>60	>60		mL/min
Glucose (random)	194	70-99	H	mg/dL
HbA1c	7.8	<5.7% n1	H	%
WBC	7.8	4.5-11.0		K/uL
Hemoglobin	14.6	13.5-17.5		g/dL
Platelets	211	150-400		K/uL
Total Cholesterol	204	<200	H	mg/dL
LDL	122	<100 (DM goal)	H	mg/dL
HDL	41	>40 M		mg/dL
Triglycerides	204	<150	H	mg/dL
TSH	2.6	0.4-4.0		uIU/mL
PT/INR	1.1	0.9-1.1		

IMAGING REPORTS:**1. CT Head Without Contrast**

Facility: Chesapeake Radiology Associates — Bowie Imaging Center

Ordered by: A. Patel MD Date: 11/12/2024

Read by: Dr. Philip H. Bernstein, MD (Radiology)

Signed: 11/12/2024 17:44

CLINICAL INDICATION: Episodic right arm weakness, headache, r/o intracranial pathology

TECHNIQUE: Axial CT images of the brain obtained without IV contrast.

FINDINGS:

No acute intracranial hemorrhage. No mass, mass effect, or midline shift. Mild periventricular and subcortical white matter hypodensities, nonspecific, likely representing small vessel ischemic changes in the setting of patient's known cardiovascular risk factors. No frank territorial infarct identified on this non-contrast study. Ventricles and sulci mildly prominent, age-appropriate. Basal cisterns patent. Calvarium intact. Visualized paranasal sinuses show mild mucosal thickening of the maxillary sinuses bilaterally, likely chronic/inflammatory.

No acute osseous findings.

IMPRESSION:

1. No acute intracranial hemorrhage or major territorial infarction.
2. Nonspecific white matter changes, likely chronic small vessel disease.
3. Mild sinus disease — clinical correlation recommended.
4. MRI brain with and without contrast recommended for further characterization of white matter findings and assessment for acute/subacute ischemia.

** MRI NOT yet obtained — see plan in Dr. Patel note **

2. Echocardiogram (TTE) — Reference: 02/2024 (prior study)

Facility: Chesapeake Heart Group

Read by: Dr. S. Gupta, MD FACC Date: 02/14/2024

[Summary only — full report in cardiology file]

EF: 58%. Mild LVH. No wall motion abnormalities. No significant valvular disease. LA dimension: 4.2 cm (mildly enlarged). No pericardial effusion.

CONCLUSION: Hypertensive heart disease pattern. Mildly enlarged left atrium.

3. EKG — 11/12/2024 (ordered, results pending at time of note)

[Awaited — not received prior to fax transmission]

[UC EKG 08/2024: NSR, rate 80, no afib, no acute ischemia — see UC note]

MEDICATION RECONCILIATION LIST

Chesapeake Regional Medical Associates

Patient: Hoffmann, Gerald T. DOB: 3/17/56 MRN: CRMA-00447821

Reconciled by: L. Nguyen, LPN Date: 11/12/2024

Source: Patient/pharmacy report — NOT fully verified

MEDICATION	DOSE	ROUTE	FREQ	NOTES
Lisinopril	10 mg	PO	QD	HTN — pt reports compliance
Amlodipine	5 mg	PO	QD	Added 2023 per cards
Atorvastatin	40 mg	PO	QHS	HLD — pt reports compliance
Metformin	500 mg	PO	BID	Pt reports D/C'd 'months ago'
Aspirin	81 mg	PO	QD	Pt reports taking 'sometimes'
Omeprazole	20 mg	PO	QD	GERD
Loratadine	10 mg	PO	QD PRN	Seasonal allergies
Vitamin D3	2000 IU	PO	QD	OTC supplement
Fish Oil	1000 mg	PO	QD	OTC — pt self-reports
Ibuprofen	400 mg	PO	PRN	Given UC 08/2024; OTC use per pt
HCTZ 12.5mg	12.5 mg	PO	QD	RECOMMENDED by cards 04/2024 — unclear if pt started

** DISCREPANCY NOTE: Cardiology note (04/2024) lists metformin as active; pt states he stopped independently. Pharmacy fill history: last metformin fill 05/2024.

Aspirin fill: last fill 03/2024. HCTZ: no fills found in pharmacy system. **

** Ibuprofen noted — use with lisinopril/amlodipine may affect BP; flagged **

ALLERGIES:

Penicillin — rash (patient-reported)

Sulfa — unknown (patient-reported)

NKDA otherwise per patient; registration notes differ from prior allergy doc

[Reconciliation incomplete — pharmacy callback requested, pending]

Signed: L. Nguyen LPN 11/12/2024 11:03

HOSPITAL DISCHARGE SUMMARY — PRIOR ADMISSION

Chesapeake Regional Medical Center — Inpatient

Patient: Gerald T. Hoffmann DOB: 03/17/1956 MRN: CRMC-881204

Attending: Dr. Kevin T. Park, MD (Hospitalist)

Admit Date: 09/28/2022 Discharge Date: 10/01/2022

Dictated: 10/01/2022 Transcribed: 10/02/2022

ADMITTING DIAGNOSIS: Acute pyelonephritis / UTI with systemic symptoms

DISCHARGE DIAGNOSIS: 1. Pyelonephritis, uncomplicated

2. T2DM — uncontrolled (A1c 8.2 at admission)

3. HTN — chronic

HOSPITAL COURSE:

Mr. Hoffmann presented via ED on 09/28/2022 with 3-day hx of fever, right flank pain, and dysuria. Temp 101.8F, BP 161/98. UA showed pyuria, bacteriuria; urine culture grew E. coli (pan-sensitive). Blood cultures negative x2.

CT abd/pelvis (09/28/2022): No abscess. No obstructive uropathy. Incidental note of mild hepatic steatosis. Prostate mildly enlarged.

Pt treated with IV ceftriaxone x 48h, transitioned to oral cefdinir at discharge.

A1c 8.2 at admission — metformin 500mg BID started (new); endocrine follow-up recommended. BP managed with home regimen + PRN IV labetalol x1.

Pt afebrile x36h prior to discharge. Tolerating PO. Ambulating independently.

DISCHARGE CONDITION: Stable / Good

DISCHARGE MEDICATIONS:

Cefdinir 300mg BID x 5 days (to complete 7-day course)

Metformin 500mg BID (new — continue outpt)

Lisinopril 10mg QD (cont.)

Atorvastatin 40mg QHS (cont.)

Aspirin 81mg QD (cont.)

Omeprazole 20mg QD (cont.)

FOLLOW-UP INSTRUCTIONS:

PCP (Dr. Patel) within 1 week — A1c management, BP check

Endocrinology referral placed — never followed up per PCP records

Repeat UA in 2 weeks

Return to ED if fever, severe pain, hematuria

Dr. Kevin T. Park, MD — Hospitalist Service

Signed: 10/01/2022 Transcription verified: 10/02/2022

[Document retrieved from CRMC medical records — faxed 11/12/2024 per patient auth]

[Note: this admission UNRELATED to current neurological complaint — included per standard records release; discharge med list may differ from current medications]

--- ADDENDUM (dictated by nursing staff, undated) ---

Patient required extra counseling re: diabetes medication compliance. Wife present

at discharge. Both verbalized understanding. Pt expressed concern about side effects of metformin; stated he may "not take it if he feels ok". Noted for PCP.

HEADACHE INTAKE QUESTIONNAIRE / NEUROLOGY PA NOTE

Chesapeake Regional Medical Associates - Neurology Satellite Clinic

(NOTE: This clinic has since closed - pt transferred back to PCP care 01/2024)

Provider: Sarah Metz, PA-C

Date: 11/07/2023 Signed: 11/08/2023

Pt: Hoffmann, Gerald DOB: 3/17/1956 MRN: CRMA-00447821

REASON FOR VISIT: "Headaches - right side - bothering me for years"

INTAKE QUESTIONNAIRE (patient-completed, scanned):

[Legibility note: original handwritten; transcribed below by MA]

Headache location: Right side, "behind eye sometimes"

Frequency: 1-2x per month

Duration: Few hours to full day

Severity: 4-6/10

Associated symptoms checked by pt: Pressure, light sensitivity (mild), nasal congestion

NOT checked: nausea, vomiting, vision changes, arm weakness, speech difficulty

Triggers identified: Stress, weather change, "not sleeping well"

Prior treatment: Ibuprofen - helps partially; tried Tylenol - less helpful

Disability: "some" - occasionally misses work activities

PA ASSESSMENT NOTE:

Mr. Hoffmann is a 67 yo M presenting to headache clinic with 2+ yr hx of episodic right-sided headaches. Based on intake questionnaire and today's interview, features most consistent with episodic tension-type headache vs possible migraine without aura. Patient specifically DENIES visual aura, nausea, and any motor or sensory symptoms with headaches (confirmed on direct questioning today).

However, of note, wife present and states: "sometimes he acts confused or his arm seems off when the bad ones hit - but he won't admit it." Patient dismisses this. Wife's observation not further elaborated. Noted for chart.

EXAM (focused):

BP 144/90, HR 80. Alert, oriented. CN intact. Strength 5/5 upper and lower extremities. No dysarthria. Gait steady. Fundoscopic: disc margins sharp b/l.

IMPRESSION: Episodic headache - probable tension-type vs migraine without aura. Low suspicion for secondary cause based on today's evaluation. Features do NOT meet criteria for hemiplegic migraine based on today's report.

PLAN:

- Trial naproxen sodium 440mg PRN for acute headache
- Headache diary - provided to patient, never returned
- Magnesium glycinate 400mg QHS as prophylaxis (OTC)
- Referral to neurology attending considered - deferred given mild sx
- Return in 3 months - pt did not follow up; clinic closed 01/2024

Sarah Metz, PA-C Supervised by: Dr. D. Krishnaswamy, MD (Neurology)

Signed: 11/08/2023 09:21

--- END OF REFERRAL PACKET ---

Total pages transmitted: 10

If all pages not received, call (301) 864-2200 - ask for referral coordinator

Fax confirm requested - please fax receipt confirmation to (301) 864-2219

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